

Editorial Review – *Transforming Healthcare*
Preface, Introduction, and Chapter 1

This is a genuinely strong manuscript: the five-pillar/dependency-matrix framework is well-designed, the OneCare autopsy is a legitimately excellent teaching case, and the "Strongest Case Against This Book's Argument" section is the kind of intellectual honesty most policy books skip. The critique below is in that spirit – it's a book worth tightening, not one with a broken foundation.

1. FACT-CHECK FINDINGS

1.1 Confirmed accurate

- Sept. 18, 2024 GMCB meeting; Oliver Wyman presented the Act 167 report; ~230 meetings, 3,100+ participants, 100+ organizations across 14 HSAs. ✓
- \$700M–\$2.4B five-year deficit range. ✓ (see 1.3 below on the "by what year" ambiguity)
- Gobeille v. Liberty Mutual Ins. Co., 577 U.S. 312 (2016), 6–2. ✓
Citation and vote count are correct.
- CBO CMMI figures: \$7.9B spent, \$2.6B in benefit-spending reductions, \$5.4B net increase, 2011–2020; 70 models tested, 4 expanded nationally. ✓ All match the CBO's September 2023 report precisely.
- Blueprint 5.8:1 ROI ("\$5.8M in reduced medical expenditure per \$1M invested"). ✓ Matches the published Blueprint/Onpoint study.
- Vermont's July 2026 AHEAD withdrawal. ✓ Confirmed – Vermont formally withdrew and shifted to the Rural Health Transformation Program.

1.2 Needs correction or a source note

- **UVMHC "417% of Medicare rates for outpatient services in 2022" (Introduction, "September 18, 2024" section). I could not find this figure in any RAND publication. The RAND figures I could locate for UVMHC are 358% (2020 study, inpatient+outpatient combined, 2018–2020 data) and 231% (2015–2017 inpatient only); RAND's most recent statewide Vermont figures (Round 5/5.1, 2022 data) put the *state* outpatient average around 263–279%, well below 417%. This is the single most load-bearing fact-check flag in the manuscript – it's cited as established fact in the opening pages, attributed specifically to RAND, and used to anchor the book's central "prices flow into premiums" argument. **Recommend**: pull the actual RAND round and table this figure comes from and cite it precisely (e.g., "RAND Hospital Price Transparency Study, Round 5.1, Table X"), or replace it with a figure you can source directly, such as the well-documented 358% (2020 study) or the GMCB's own FY25 Hospital Budget Order language about UVMHC's price decile.

1.3 Internal inconsistency: the \$195M RHT figure

- Chapter 1, §1.11.1: "The **\$195 million** Rural Health Transformation award is a Policy-pillar outcome" – read as a single, total award.
- Chapter 1, §1.11.3: "the five-year, **\$195-million-per-year** Rural Health Transformation Program" – read as an *annual* figure (implying ~\$975M over five years).
- Public reporting (WCAX, July 2026) confirms the correct figure is **\$195 million per year for five years** (~\$975M total to Vermont).

§1.11.1's phrasing needs to be fixed to match §1.11.3 – as written, a reader who only reads the earlier passage will walk away thinking Vermont's total RHT award is \$195M, which understates it roughly fivefold.

1.4 Worth a source note (not necessarily wrong, but unverifiable from public reporting as stated)

- **"\$700 million to \$2.4 billion... by [year]"**: the book never states the year in the Preface or Introduction, and that's actually the right call – press coverage of the same Oliver Wyman figure is inconsistent (VTDigger has reported "by 2028," "by 2029," and "by 2030" in different articles). Worth adding a footnote acknowledging this reporting variance so a skeptical reader doesn't think *you* introduced the ambiguity.

- **OneCare's internal data lag** ("A precise day-count... is not independently documented in the public record"): commendable that the book flags this itself rather than inventing a number – leave as is.

- **Median age ranking** (Preface, "Beyond Vermont" box: VT 4th-oldest at 42.8, behind Maine 45.1, NH 43.0, WV 42.9): I did not independently verify these specific figures against Census/ACS vintage data. Given how central this claim is to the "Vermont is a preview" argument, it's worth double-checking the vintage year against the source cited and confirming NH's rank, since New Hampshire's median age is frequently reported closer to Vermont's than this ranking suggests.

- **Administrative cost figures**: UVMHC "\$3,826 against a \$1,427 benchmark" (§1.4.4) and, separately, Vermont's "critical-access and rural hospitals" at "\$2,730 per discharge... compared to a national benchmark of \$1,427" (§1.11.6) – these describe two different hospital subsets against the *same* \$1,427 benchmark. That's plausible (different peer groups can share a national benchmark), but because the two figures appear ~200 pages apart in different contexts, a reader may reasonably wonder if they're the same statistic misremembered. A one-line clarification in one of the two places ("as distinct from UVMHC's per-discharge administrative cost cited in §1.4.4...") would remove the doubt.

1.5 Note on citing future events as settled fact

Several passages (the AHEAD withdrawal discussion, §4 and §8 of "The Strongest Case," §1.14) state outcomes of events (Vermont's AHEAD withdrawal, the UVMHC court loss, specific FY23 GNCB enforcement dollar amounts) as already-resolved fact. These check out against public reporting as of this review. Given how quickly this specific policy landscape is moving (three enforcement actions, a federal withdrawal, and program cuts all inside about 18 months), it's worth a final pre-publication pass on every dated claim in these sections immediately before the book goes to print, since some of this is likely to keep changing.

2. STRUCTURAL / REDUNDANCY ISSUES

2.1 The "why equity isn't a sixth pillar" argument is stated in full four separate times

The identical argument – a sixth pillar would have its own budget/staff/queue position and could be deferred; an imperative can't be deferred because there's no separate project to defer – appears nearly verbatim in:

1. Preface (§ beginning "That last failure is why equity does not appear...")
2. Introduction, "The Five-Pillar Framework and the Equity Imperative" section
3. Chapter 1, §1.3 ("Equity is different in kind...")
4. Chapter 1, §1.4.5 opening line

Each repetition uses close paraphrase of the same two sentences ("its own budget line/staff/queue... which means it could be sequenced last, funded least, reported separately"). This is the single biggest redundancy in the manuscript. **Recommend**: state it in full once (the Introduction is the right place, since it's the book's first real argument-bearing section), and in the Preface and both Chapter 1 locations, compress to a single sentence with a forward/backward reference ("as the Introduction explains, equity is an imperative rather than a sixth pillar because...").

2.2 The OneCare "three sequencing failures" are narrated three times at increasing depth

This one is more defensible – Preface (glancing mention), Introduction (brief), Chapter 1 §1.2 (full autopsy), §1.9 (recap), §1.11 (resolved-sequence recap), §1.13 (diagnostic table recap) – but by the fifth or sixth pass through "voluntary participation, no mandate, BCBS exit, Managing Blind," the specific phrase **"Managing Blind"** and the **93,000 enrollees / one-third** figure are each repeated close to six times word-for-word across the chapter. This is a case where the repetition is pedagogically intentional (it's a teaching device – return to the same case at increasing resolution), but a few of the later recaps (particularly the one embedded in §1.13's table and the one in §1.11.1) could be tightened to a fragment ("OneCare's voluntary-architecture failure, above") without losing the pedagogical thread.

2.3 "BEYOND VERMONT" and "VERMONT EVIDENCE" boxes are doing overlapping work in places

Some boxes (e.g., the one following Policy→Economics in §1.4.1, and the Technology→Economics "Beyond Vermont" box in §1.4.2) restate a claim already made in the surrounding prose almost verbatim before adding the actually-new information (the Maryland comparison, the 24-state APCD stat). Tightening these so the box opens directly with the new fact, rather than re-summarizing the paragraph above it, would save real length without cutting content.

2.4 The "gate" metaphor is defined twice

The "WHAT A 'GATE' MEANS IN THIS BOOK" box (§1.12.1) is excellent and should stay – but "gate," "open," and "closed" are already used with the same meaning as early as §1.9 and the Worked Example in §1.4.3 ("The Technology gate was closed before she signed"), well before the formal definition arrives. Consider moving the definition box earlier (to §1.4, where the Dependency Matrix is introduced) so the vocabulary is

established before it's used, rather than used first and defined ~30 pages later.

3. PASSAGES THAT ARE HARD TO FACT-CHECK OR OVER-CLAIM PRECISION

- **"Roughly 24 states operate an established all-payer claims database"** (§1.4.2, "Beyond Vermont" box): plausible (APCD Council figures typically cite 20s), but stated without a source note in a book that is otherwise careful to source specific numbers. Add a citation or soften to "roughly two dozen."

- **"41% of rural hospitals operate in the red and roughly 768 are at risk of closing, 315 of them within three years"** (Preface, "Beyond Vermont" box): these numbers track closely with Chartis Center for Rural Health figures that circulate frequently in health-policy writing, but the source isn't in the sources list at the end of the Introduction or the chapter. Given how much weight the Preface puts on this national-comparison claim, it should be sourced explicitly, not just asserted.

- **"Vermont's 91% primary care access rate... four points above the national benchmark... 11-point gap between white and BIPOC adults"**: repeated four times (Introduction, §1.3, §1.4.2, §1.11.5) with identical figures each time – good consistency – but the underlying Vermont Department of Health Health Equity Data Report is only cited once (§1.4.2, "Vermont Evidence" box). Consider citing it at first use (Introduction) rather than at its third occurrence.

4. PROSE / STYLE NOTES

- **Overuse of the em-dash sentence fragment as a rhetorical device.** This is a genuine stylistic signature of the book (e.g., "Not aspirational change. Not incremental improvement. But actual change...") and used well in a handful of key places (the Preface's opening, the OneCare autopsy's close). But by Chapter 1 it recurs so often – nearly every subsection has at least one bolded, one-sentence aphorism set off on its own line ("A policy intervention that passes question one but fails questions three, four, or five is not a solution – it is a well-intentioned mistake waiting to be made") – that the device loses force. Recommend cutting roughly a third of these standalone bolded aphorisms, keeping the strongest 2-3 per chapter rather than 1-2 per subsection.

- **"Structural, not rhetorical"** and close variants ("architecture, not preference"; "structural rather than a matter of effort, funding, or political will"; "not a preference, an architecture") appear as a rhetorical pattern at least eight times across these three sections. It's a good line the first two or three times; by the sixth repetition it reads as a verbal tic rather than emphasis.

- **The phrase "is not X. It is Y."** (two-sentence antithesis) is used extremely frequently – dozens of instances across ~20,000 words. This is a deliberate rhetorical register for a book aimed at policy professionals, and works well in moderation, but at this frequency it

starts to read as formula rather than argument. A pass specifically hunting this construction and varying roughly every third instance would improve the prose noticeably without touching the substance.

- **Chapter 1's opening abstract** (the italicized paragraph under the chapter title) and §1.1's opening paragraph both introduce the same claim – that healthcare transformation is a "system problem" requiring "system solutions" – within about 300 words of each other. Minor, but tightenable.

- The bolded pull-quote **"Healthcare transformation is a system problem, and system problems require system solutions"** in §1.1 restates the chapter abstract almost word for word ("Healthcare transformation is a system problem: five pillars..."). Consider varying the phrasing of one of the two.

5. CONTENT THAT COULD BE CUT OR MOVED WITHOUT LOSS

1. **The second half of §1.6** ("Using the Dependency Map as an Analytical Tool"), items 2 and 3 ("Risk identification" and "Investment prioritization"), largely restates conclusions the reader has already reached independently by this point in the chapter (that Technology is the highest-leverage/highest-risk pillar). This section could be compressed to a single paragraph, or cut, since §1.14's timeline table and §1.14.1 make the same "Technology is the current bottleneck" point with more specific, dated evidence.

2. **The "How to Use This Book" reader-profile sections** (four full profiles: Policy Professional, Executive, Vermont Practitioner, Student/Researcher) are useful, but each profile's "In the Academy" and "On the platform" paragraphs are close to interchangeable in structure and could be tabulated (as the HTR Ecosystem table already does elsewhere in the Introduction) rather than written out in full prose four times. This would cut roughly 40% of the section's length with no loss of information.

3. **"What This Book Cannot Do"** and the Preface's "This book is also not a prediction" paragraph make substantially the same disclaimer (the book can't predict the outcome, can't substitute for expertise, can't resolve politics) in two different places about 15 pages apart. One could be trimmed to a cross-reference.

6. WHAT'S WORKING WELL (worth protecting through any edit pass)

- The **"Strongest Case Against This Book's Argument"** section is the strongest single piece of writing in this excerpt – it's rare for a policy book to engage its own best counterarguments this directly, and the AHEAD-skepticism argument in particular reads as genuinely prescient given the July 2026 withdrawal.

- The **OneCare autopsy** (§1.2) is a legitimately well-constructed case study: it correctly separates a legal cause (Gobeille), an institutional cause (BCBS exit), and an internal-capacity cause (analytics lag), and its "cascade" table is a clean, checkable causal chain.

- The **"What a Gate Means in This Book"** and **"Worked Example – Signing a Contract You Cannot Read"** boxes are the two best pieces of applied writing in the chapter – concrete, memorable, and doing real explanatory work rather than restating the abstract framework.

7. PRIORITY ACTION LIST

1. **Fix or source the UVMMC 417%/RAND figure** – highest-priority fact-check item; it's cited as an established fact in the book's opening pages.
2. **Reconcile the \$195M RHT figure** (total vs. per-year) between \$1.11.1 and \$1.11.3.
3. **Cut the "sixth pillar" argument down to one full statement + three short cross-references.**
4. **Source the "24 states have an APCD" and "41%/768/315 rural hospitals" claims explicitly**, since they're currently asserted without citation in a book that sources almost everything else.
5. **Do a global search-and-thin pass on "is not X. It is Y." constructions and standalone bolded aphorisms** – keep the best, cut roughly a third.
6. Before final print, **re-verify every dated claim tied to 2026 events** (AHEAD withdrawal, GMCB enforcement dollar figures, UVMMC litigation outcome), since this is a fast-moving live policy environment and several of these were current only as of mid-2026.

Happy to go deeper on any one of these – for instance, I can mark up specific passages with suggested rewrites, or do a focused pass on later chapters if you'd like the same treatment applied there.